CHIROPRACTIC
SLEEP ACADEMY

CHIROPRACTIC SLEEP ACADEMY®

How Dentists Increase Chair-Hour Efficiency in Sleep Apnea Cases

By Working With Doctors of Chiropractic — A Practical Workbook for Dentists

Dr. Robert Zeravica, DC
Chiropractic Sleep Academy®
www.ChiropracticSleepAcademy.net

"You don't have to run the playground. You just need to know where to stand."

INTRODUCTION

The Playground

A new kid joins the school playground. The dentist and the Doctor of Chiropractic both want to help. Someone has to explain the rules.

This workbook explains what the playground is (sleep apnea care), who sets the rules (biomechanics + scope), and how dentists can **help more patients in less chair time** — without turf wars, over-education, or wasted effort.

PART 1

The Opportunity — and the Bottleneck

Sleep apnea patients are entering dental offices more than ever. Dentists are often the **first** to open the airway conversation — you screen airway, CBCT gives visual authority, and the first apnea conversation feels natural in a dental setting. That's not about ownership; it's about **patient acceptance.**

But airway cases can quietly eat chair hours:

- appliance titration that stalls,
- intolerance and remakes,
- TMJ complaints after delivery,
- patients who "don't feel better" despite a well-made device,
- documentation and follow-up that pile up.

Most of these aren't device problems — they're **global-mechanics problems** the device alone can't solve.

PART 2

Why CBCT and the Appliance Alone Aren't Enough

CBCT shows airway narrowing, jaw position, craniofacial anatomy — a **local** view. It does not show spinal-curve load, pelvic imbalance, respiratory mechanics, postural compensation, or foot–spine relationships.

> *A narrow airway is a local finding. Sleep apnea is a global condition.*

When the global frame isn't addressed, an oral appliance is placed on a body that may be fighting it — the source of intolerance, relapse, and TMJ strain. The device isn't wrong; it's **unsupported.**

PART 3

What the Doctor of Chiropractic Adds

A post-graduate-trained DC evaluates the whole structural frame the airway sits in: posture / cervical curve (CBP); pelvis & SIJ (SOT/AK); cranial mechanics (BNS/CFR); spinal loading; foot–pelvis–spine integration. This answers one question that protects your appliance work:

> *Will an oral appliance help — or create new problems — without correcting global mechanics?*

The DC doesn't touch your scope. You choose and deliver the device; the DC makes sure the body can accept and hold it.

WHERE THE LANES MEET

The TMJ — and the Joint That Finishes It

In the ideal case, the **TMJ is the cross-over joint** between you and the Doctor of Chiropractic. Both of your licenses cover it, so it's the natural front door for working together — the one joint where "everyone in their lane" becomes "and here's where the lanes meet."

Here's the part most sleep programs miss. In the chiropractic biomechanical model, the **TMJ and the sacroiliac joint (SIJ) behave as a reciprocal pair** — top and bottom of the spine, moving in relationship. In that model, **TMJ work done without addressing the SIJ is incomplete.** You work the TMJ; the Doctor of Chiropractic owns the SIJ (SOT/AK) — the TMJ's paired partner. That's not a courtesy referral; it's finishing the job at both ends of the same system.

> *The dentist works the TMJ. The Doctor of Chiropractic finishes it at the SIJ.*

A note on coordination (California). Because a California Doctor of Chiropractic's scope covers the whole skeleton — skull to sacrum to feet — the DC is well-placed to serve as the **case manager of the whole-body sleep case** while you run the oral-appliance part of the play. This isn't about rank; it's about who holds the whole-body view. Sleep studies stay physician-read, and moderate-to-severe disease is co-managed with the sleep physician. *(Scope described here is California-specific.)*

PART 4

The Proper Role of the Oral Appliance

Oral appliances **work** — they support airway patency and reduce collapse, and for many patients they're the right tool. Selection is best made as a **co-decision:**

- **DDS** evaluates dental tolerance (occlusion, perio, restorations, TMJ).
- **DC** evaluates biomechanical consequences (posture, cervical load, cranial/pelvic response).

Both providers trained on the device. The appliance **supports the structure — it doesn't replace it.**

PART 5

The Ideal Sequence

1. **DDS** identifies the airway concern.
2. **DDS** refers to the **DC** for whole-system evaluation.
3. **DC** evaluates biomechanics and reports back.
4. **DDS + DC** select the appliance together.
5. **DDS** delivers the appliance.
6. **DC** reassesses global mechanics post-delivery.
7. Adjustments/supports as needed; **retest (physician-read HST) at ~30–60 days.**

No turf war. No duplication. No guessing.

PART 6

The Chair-Hour Math

Co-management doesn't add work — it **removes** the chair hours airway cases usually cost:

- **Fewer remakes and titration stalls** — the body is prepped to accept the device.
- **Fewer TMJ/intolerance call-backs** — cervical and cranial load addressed up front.
- **Faster patient acceptance** — the patient hears a coordinated, whole-body plan.
- **Shared documentation** — the DC's findings and re-tests support your chart.
- **Better, measurable outcomes** — pre/post HST numbers you can both stand behind.

PART 7

Protection — Why This Covers Everyone

- **Dentists** reduce chair time and, through documented co-management, may reduce liability exposure. *(Have your attorney confirm any liability language.)*
- **Doctors of Chiropractic** practice within scope.
- **Manufacturers** stay in their lane.
- **Patients** get better outcomes.

Sleep studies remain **physician-read**; moderate-to-severe disease is co-managed with the sleep physician. Everyone stays in their lane; the case gets coordinated.

PART 8

How to Start

1. **Find a CSA-trained DC** (or one skilled in CBP/SOT/AK/cranial) near you.
2. **Agree on the sequence** and a simple referral pathway (form + shared summary).
3. **Pilot 3–5 cases** with pre/post HST so you both see the difference.
4. **Standardize** the co-decision checklist for every airway patient.

Checklists & Roles

A. Referral snapshot (DDS → DC)

Patient, airway concern, CBCT note, appliance considered, question for the DC ("can the body accept/hold this device?").

B. Co-decision checklist (appliance selection)

- ☐ DDS: occlusion / perio / restorative tolerance
- ☐ DDS: TMJ baseline
- ☐ DC: cervical curve & posture load
- ☐ DC: pelvic / cranial response
- ☐ Joint: device choice + titration plan
- ☐ Plan: physician-read HST pre & ~30–60 days post

C. Who does what

Step	DDS	DC	Physician
Screen / open conversation	●	○	
CBCT / dental airway view	●		
Whole-system biomechanics		●	
Sleep study read			●
Appliance choice	● (co)	● (co)	
Appliance delivery	●		
Post-delivery reassessment	○	●	
Moderate–severe management	co	co	●

D. Red flags → physician now

Severe daytime sleepiness with driving risk, witnessed apneas with cardiac history, oxygen-desaturation concerns — route to the sleep physician promptly. Structural care is co-managed, not a substitute.

> *Dentists open the door. Doctors of Chiropractic assess the whole house. Oral appliances support the structure — they don't replace it.*